

Our Ref: (T) RH/AS/ 0556486

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

Joint IBD Clinic

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

Room 145, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME]

Lingfield, [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Pan-ulcerative colitis diagnosed 2010

Type 2 diabetes on insulin

CKD

Bile salt malabsorption

Mental health issues

Medications : Mercaptopurine 25 mg OD, Allopurinol 100 mg od.

I reviewed Valerie in our IBD Clinic today. Her bowels are opening four or five times per day stopped her Cholestagel therapy as it was causing severe constipation. I have discussed that try very low dose cholestagel or cholestyramine if she did wish to reduced her bowel motion f Angela tells me she is having regular blood tests with yourselves in Salisbury due to her ren and hyperkalaemia. Indeed the potassium on blood tests done yesterday was 5.5.

With regards to her bowels things are fairly stable she opens them between four and five time [NAME] has no rectal bleeding. Her blood tests show a picture of anaemia of chronic disease w of 23. I note her most recent endoscopy showed very little in the way of active disease and i CRP relates to her adiposity.

[NAME] has been no changes to her medication but we will see her again in 6 to 12 months' ti

Yours sincerely

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[PROVIDER-NAME]

IBD Research Fellow to [PROVIDER-NAME]